GEEVI EVIDENCE BRIEF - PUBLISHED SOURCE - ATTRIBUTED SUMMARY

ELN 2022 AML recommendations - evidence brief

Source type: Expert-panel recommendations

Publication date: 2022-09-22

Citation: Döhner H, Wei AH, Appelbaum FR, et al. Diagnosis and management of AML in adults: 2022 recommendations from an international expert panel on behalf of the ELN. Blood. 2022;140(12):1345-1377. doi:10.1182/blood.2022016867. PMID: 35797463. https://pubmed.ncbi.nlm.nih.gov/35797463/

Question: Which diagnostic, genetic-risk and response-assessment information should be reviewed before an AML case is discussed under a departmental pathway?

Population: Adults with AML; this is an expert recommendations document, not a randomized patient cohort.

Intervention: Not applicable: recommendations and classification framework.

Comparator: Not applicable: no experimental treatment comparison.

Outcomes: Updated disease and genetic-risk classification Genomic diagnostics and measurable residual disease assessment Revised response criteria and management recommendations

Finding 1: The 2022 panel update addresses advances in molecular classification, genomic diagnostics, measurable residual disease and AML therapies. Its abstract identifies revisions to genetic-risk classification and response criteria; it is a reference for documenting the framework used in a case assessment, not an automatic classifier.

Limitations: The PubMed citation and abstract were verified. The publisher full-text page returned a security-verification 403, so detailed tables and workup rules were not independently reviewed in this research pass. Published in 2022. This brief does not establish that the source is the latest guideline or that one risk framework applies to every treatment context. A recommendations document does not supply an individual patient's diagnosis, fitness assessment or treatment decision.

Applicability: Use as a sourced discussion prompt for an AML profile: diagnosis/classification basis, relevant molecular information, response assessment and unresolved inputs. The oncologist must record the actual assessment and chosen framework. Unknown or pending fictional results remain unknown or pending. A departmental protocol derived from this source still requires local scope, independent review, approval and a review date.

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Original Geevi evidence brief, not the original article or a treatment recommendation.

Sources verified: 2026-10-11. Full-text appraisal and a current literature review remain necessary.